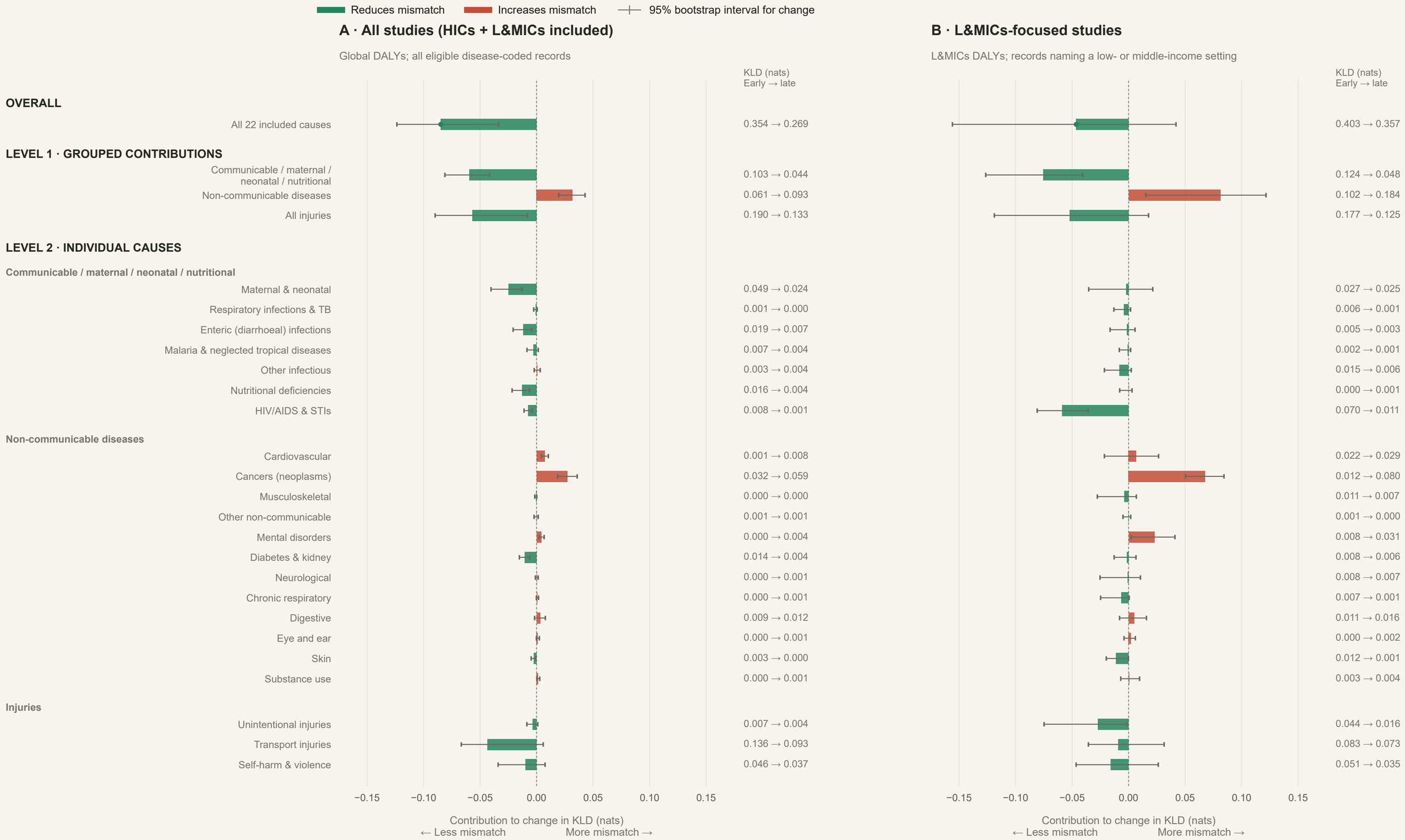
KLD CONTRIBUTIONS: 22-CAUSE EXTENDED ALL-CAUSE SET

2010–2012 → 2021–2023: research and DALY distributions both change, as in Fig. 2a.
Overall, level-1 and level-2 sections show the same change at increasing detail.



Overall: 0.354 → 0.269; change -0.085 nats
95% interval -0.124 to -0.033; n = 3,692 → 7,822

Overall: 0.403 → 0.357; change -0.046 nats
95% interval -0.156 to +0.042; n = 518 → 2,005

L&MICs = low- and middle-income countries (low-, lower-middle- and upper-middle-income groups). 22 GBD level-2 causes; DALY shares p and study-cause-link shares q are separately normalised over this set. KLD = sum p ln(p/q), in nats. Nonnegative disease contributions use p ln(p/q) - p + q; their sum equals KLD. This disease allocation differs from signed p ln(p/q) terms, although the overall total is identical. Level-1 rows sum descendant contributions, retaining within-group mismatch; do not add across sections. Both research and burden change, so the change cannot be attributed solely to research. Intervals: 1,000 study bootstrap draws within year; DALY/coding uncertainty omitted. Zero-count causes receive 0.5 only when needed, following the existing Fig. 2a implementation; details in provenance. Common ordering: level-1 groups, then global DALYs summed over 2010-2023. Panel A includes unknown-country records; Panel B includes mixed-income studies naming an L&MICs setting. Research uses publication-year income; burden uses burden-year income. Latest window includes the COVID period. The 16-cause set excludes mental disorders, all injuries, other infectious and other non-communicable diseases; the 22-cause extension includes them. These are adaptations to economic-evaluation records, not exact replications of the reference sample.